

Medicaid/CHIP Enrollment Divergence After Unwinding, 2019-2026

Caroline Howard | June 24, 2026

Interactive dashboard: <https://d54fbb87-f0fd-4ddf-a3b5-56732692c752.plotly.app/>

Methods and code: <https://github.com/caroline-howard/medicaid-enrollment-policy-dashboard>

Overview

After the pandemic-era continuous coverage period ended, Medicaid/CHIP enrollment did not move uniformly across programs or states. This brief uses official public monthly reporting from January 2019 through February 2026 for all 50 states and the District of Columbia to summarize where enrollment stayed above baseline, where it fell below baseline, and which trajectories warrant closer review after unwinding.

National Trend

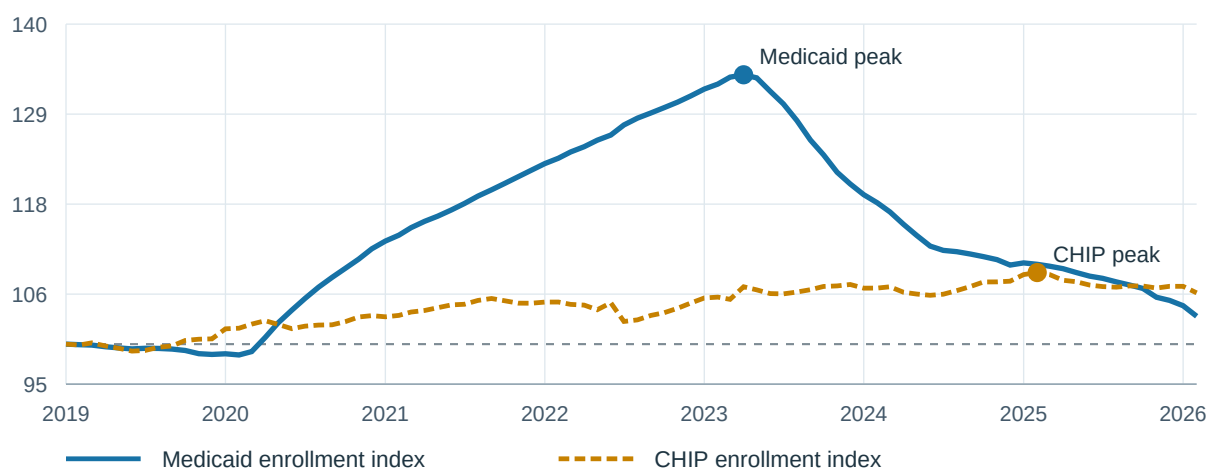
Total Medicaid/CHIP enrollment rose from 71.4 million in February 2020 to 94.7 million at its April 2023 peak, then declined to 74.9 million by February 2026, a 20.9% decline from peak. Even after that decline, enrollment remained 3.5 million people, or 4.8%, above its February 2020 pre-pandemic level (Centers for Medicare & Medicaid Services [CMS], 2026).

Medicaid drove most of the post-peak contraction. Medicaid enrollment rose from 65.4 million in January 2019 to 87.4 million at its April 2023 peak, then declined to 67.7 million by February 2026, a 22.6% decline from peak but still 3.5% above the January 2019 baseline. CHIP followed a different pattern: enrollment increased from 6.8 million in January 2019 to a later February 2025 peak of 7.4 million, then declined only 2.3% to 7.2 million by February 2026, remaining 6.4% above January 2019.

This timing difference should be interpreted in the context of post-pandemic eligibility rules: Medicaid renewals resumed after the federal continuous enrollment condition ended in 2023, while children's coverage was also affected by 12-month continuous eligibility protections in Medicaid and CHIP beginning in 2024 (CMS, 2023a, 2023b).

Children's coverage also declined sharply from its pandemic-era high but did not fall below the February 2020 pre-pandemic baseline in these data. Child Medicaid/CHIP enrollment rose from 35.3 million in February 2020 to 42.3 million at its April 2023 peak, then declined 15.7% to 35.7 million by February 2026. By the latest month, child enrollment remained 374,531 higher than February 2020, or 1.1% above baseline (CMS, 2026).

Figure 1. Indexed Medicaid and CHIP Enrollment Trends, January 2019-February 2026

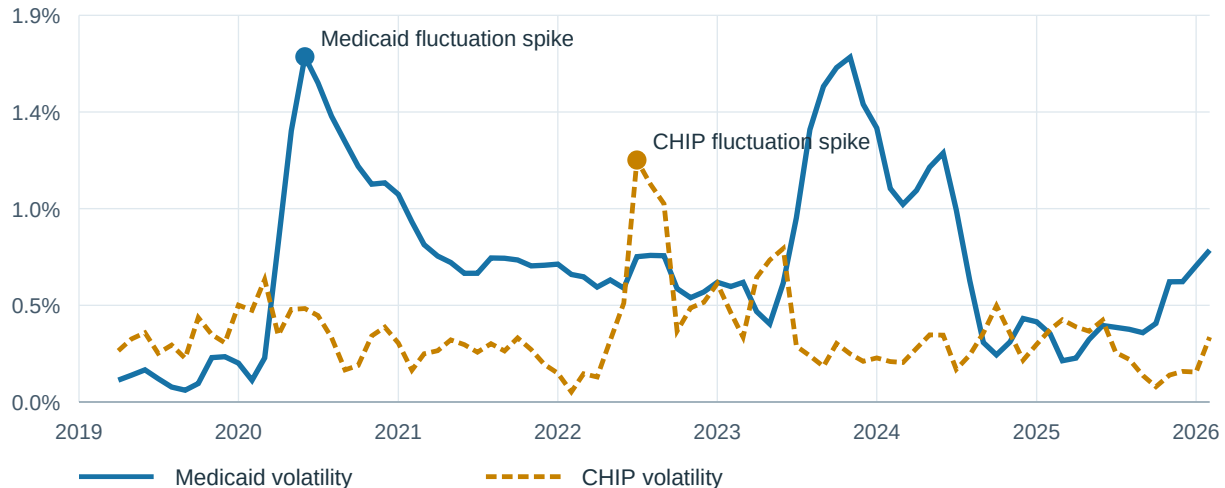


Indexing both programs to January 2019 = 100 makes timing and relative movement visible despite different enrollment scales.

As shown in Figure 1, the indexed trend clarifies the difference between the two program trajectories. Medicaid declined 22.6% after its April 2023 peak, while CHIP peaked later, in February 2025, and declined 2.3% by February 2026. That timing difference indicates the post-2023 enrollment decline was concentrated more heavily in Medicaid than in CHIP.

To assess whether the two programs also differed in operational instability, the volatility measure uses a 3-month rolling average of absolute month-to-month percent change to compare Medicaid and CHIP enrollment volatility on the same scale.

Figure 2. Rolling Volatility in Medicaid and CHIP Enrollment, January 2019-February 2026



The volatility measure uses a 3-month rolling average of absolute month-to-month percent change.

As shown in Figure 2, Medicaid enrollment was more volatile than CHIP enrollment across the period, with the largest short-term movement during the early pandemic period and the post-2023 decline. CHIP was steadier overall, though isolated spikes still appeared. For policy review, these short-term movements may signal operational disruption, reporting changes, or concentrated renewal processing rather than gradual eligibility change alone. Together, the level and volatility views raise the state-level question of which states diverged most from the national pattern, nearby peers, or their own earlier trajectory.

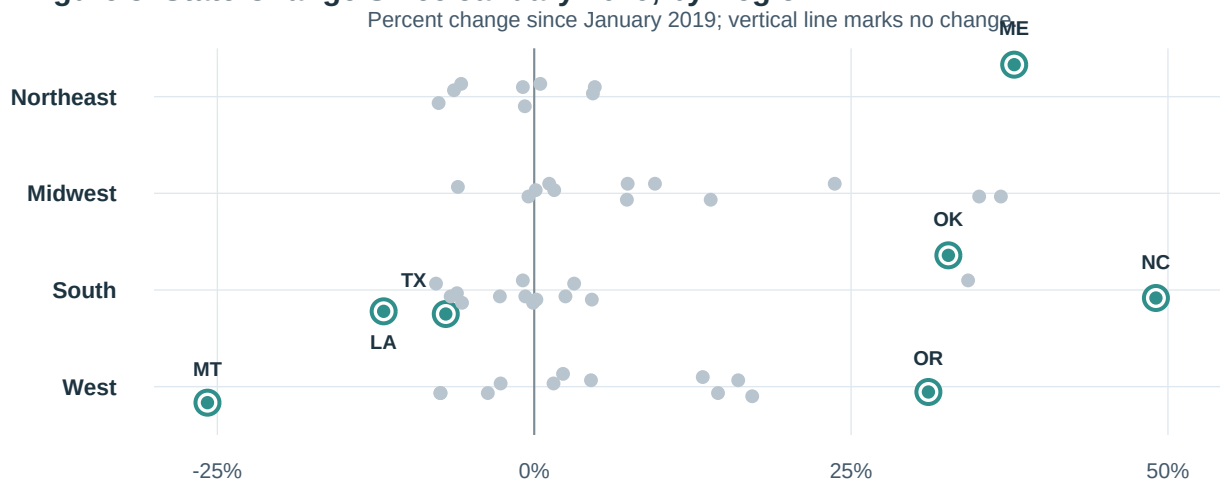
State Divergence from the National Pattern

Using the January 2019 dashboard baseline, 21 states and DC were below baseline by February 2026. State changes ranged from a 25.8% decline in Montana to a 49.1% increase in North Carolina. Using February 2020 as a pre-pandemic sensitivity check, 19 states and DC were below baseline by February 2026, with changes ranging from a 17.6% decline in Montana to a 53.1% increase in North Carolina (CMS, 2026).

Regional comparisons show that nearby states often moved in different directions. In the South, North Carolina was 49.1% above January 2019 while Texas (-7.0%) and Louisiana (-11.9%) were below baseline; Virginia (+34.2%) and Oklahoma (+32.7%) were also more than 30% above baseline. In the Northeast, Maine (+37.9%) separated from the rest of the region as an upward outlier. In the Midwest, Missouri (+36.8%) remained above baseline while Michigan (-6.0%) moved below it. In the West, Oregon (+31.1%) remained above baseline while Montana (-25.8%) had the largest decline in the country. These contrasts identify states whose trajectories diverged enough from nearby peers or from the national pattern to warrant closer follow-up.

Figure 3 shows these regional contrasts by displaying which states remained above or below their January 2019 enrollment baseline and which outliers stand apart from nearby states.

Figure 3. State Change Since January 2019, by Region



The regional distribution highlights nearby states with sharply different trajectories, including Maine in the Northeast, North Carolina and Oklahoma in the South, and Montana and Oregon in the West.

Within-State Anomalies and Turning Points

For state-level follow-up, the most useful cases are not simply the states with the largest declines, but the states whose later trajectories changed character relative to their own earlier history. Peak-to-current change helps separate scale from trajectory by identifying abrupt shifts within each state.

Montana and Texas illustrate unusually steep post-peak contraction, with latest enrollment far below each state's observed enrollment peak. Florida and Michigan point to a different trajectory: delayed reversion below the January 2019 baseline after earlier pandemic-era elevation. These distinctions matter because they point to different possible explanations. Sharp post-peak contraction may reflect unwinding intensity, renewal processing dynamics, or administrative/reporting issues. Delayed reversion below baseline may indicate a slower but more durable shift in eligibility, take-up, administrative practice, or policy context. The strongest follow-up targets are therefore the states where multiple signals overlap - especially where decline from peak, reversion below baseline, and divergence from nearby peers all appear at once.

Conclusion

Three findings warrant further review. First, national Medicaid/CHIP enrollment was still 3.5 million people, or 4.8%, above February 2020 by February 2026, but it had declined 19.8 million, or 20.9%, from the April 2023 peak. Second, Medicaid and CHIP followed different timelines: Medicaid peaked in April 2023 and declined 22.6% by February 2026, while CHIP peaked later, in February 2025, and declined 2.3%.

Third, state-level divergence remained large under both baselines: 21 states and DC were below January 2019 levels, while 19 states and DC were below February 2020 levels. Montana had the largest decline under both comparisons, while North Carolina had the largest increase (CMS, 2026).

Montana, Texas, Florida, and Michigan combine sharp peak-to-current decline with reversion below baseline, making them stronger candidates for state-specific policy review than states with only one anomalous indicator. A next-stage analysis should compare these states' unwinding timelines, renewal procedures, ex parte renewal performance, administrative reporting notes, and relevant eligibility policy changes to assess which operational or policy factors are most plausibly associated with divergent enrollment outcomes.

References

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